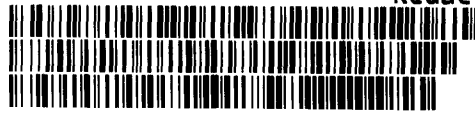


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Final Surgical Pathology Report

Procedure:

Diagnosis

- A. Sentinel lymph node, right axilla, biopsy: Two negative lymph nodes (0/2).
- B. Breast, right, excisional biopsy:
 - Invasive ductal carcinoma, grade 2, size not less than 2.5 cm, present at the medial and posterior margins; see microscopic description.
 - Ductal carcinoma in-situ, grade 2, present at the inferior margin.

Microscopic Description:

Invasive Carcinoma:

Histologic type: Ductal
Histologic grade: 2
Overall grade: 7/9
Architectural score: 3
Nuclear score: 2
Mitotic score: 1

Greatest dimension (pT2): Not less than 2.5 cm, and possibly as much as 4.0 cm (invasive carcinoma extends away from the main tumor mass into the surrounding fat throughout much of the specimen).

Specimen margins: Invasive carcinoma is broadly present at the medial and posterior margins.

Vessel invasion: Not identified

Calcification: Not identified

Ductal carcinoma in situ:

Histologic pattern: Solid, cribriform
Nuclear grade: 2
Central Necrosis: Focal
% DCIS of total tumor (if mixed): 2%
Extensive intraductal component (present/absent): Absent
Specimen margins: DCIS is at the inferior margin of the specimen

Calcification: Negative

Comments: Two prior biopsy sites are identified in the mid-portion of the specimen. There is a confluence of tumor present microscopically between the two apparent masses formed by the tumor, supporting the conclusion of a single tumor mass lesion. Additionally, there is tumor infiltrating around and beyond the tumor mass areas, into the surrounding fat, and to the specimen margins as stated above. p63 immunostaining performed to confirm the presence of invasive carcinoma at the margin. E-cadherin performed to exclude LCIS and confirm DCIS at the inferior margin.

Cytokeratin immunostaining performed on two lymph node blocks to exclude metastatic carcinoma; stains are negative.

Prognostic markers: See previous biopsy report

1CD-0-3

carcinoma, infiltrating duct, NOS 8500/3
Site: breast, NOS 050.9

hw
10/27/12

[A few of the antibodies used in our laboratory may be classified as analyte specific reagents. These antibodies are monitored and controlled in our laboratory and their performance for in vitro diagnosis is well described in the medical literature. They have not been cleared or approved by the FDA.]

Specimen

- A. Right axillary sentinel lymph node
- B. Right breast mass, long anterior, short superior - tissue procurement

Clinical Information

Right breast cancer; year old black female with two side by side cancers

Gross Description

- A. Received unfixed, labeled right axillary sentinel node, are 2 lymph nodes, 0.7 and 0.8 cm in greatest dimension each, entirely submitted with each lymph node and a separate block (bisected).
- B. Received unfixed, and a transparent container, labeled right breast mass, is a 5.5 x 4.3 x 3.0 cm portion of fibroadipose tissue with orienting sutures. Margins are inked: Anterior black, posterior blue, lateral green, and medial yellow. The specimen is serially sectioned from superior to inferior. There is a 2.5 x 2.5 x 1.5 cm tumor mass, which is somewhat dumbbell-shaped, but appears to consist of one tumor mass. Sections submitted as follows: 1 superior margin, 2 through 6 superior-most portion of tumor, 7 through 9 transition area of probable second lobe of tumor mass (contiguous sections), 10 breast tissue intervening between tumor mass and margin, 11 - inferior margin. Portion of tumor and normal submitted for tissue procurement, as requested. Additional sections submitted: RR1 intervening tissue between superior margin and tumor; 2-4 additional superior tissue, 5-7 representative remaining first half of specimen; 8-14 inferior half of specimen; 15 additional inferior margin.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
Metastasis Discrepancy		
Pathologic History		
Quality of Specimen		
Quantity of Specimen		
Container Labeling		
Date Reviewed:		

10/27/12